

TYPES OF CANCER

Prostate cancer



PROSTATE CANCER BOOKLET: EVERYTHING YOU NEED TO KNOW

What is prostate cancer?

Prostate cancer develops when abnormal cells in the prostate gland grow in an uncontrolled way, forming a malignant tumour.

Symptoms

Diagnosis

Treatment

JUMP TO SECTION:

- ▶ ABOUT PROSTATE CANCER
- ▶ SYMPTOMS
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- ▶ DIAGNOSIS
- ▶ TREATMENT

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Understood



Prostate cancer is the most common cancer in Australian men (except for skin cancer). About one in 12 men will get prostate cancer by the age of 70, and one in five will get it in their lifetime.

Early (localised) prostate cancer refers to cancer cells that have grown but do not appear to have spread beyond the prostate.

There are two stages of advanced prostate cancer:

locally advanced prostate cancer where the cancer has spread outside the prostate to nearby parts of the body or glands close to the prostate

metastatic prostate cancer where the cancer has spread to distant parts of the body.

► **PREVENTION**

► **PROGNOSIS**

Prostate cancer signs and symptoms

Early prostate cancer usually does not cause symptoms.

Advanced prostate cancer symptoms can include:

frequent urination

pain while urinating

blood in the urine or semen

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weak legs or feet

More widespread disease often spreads to the bones and causes pain or unexplained weight loss and fatigue.

Causes of prostate cancer

Some factors that can increase your risk of prostate cancer include:

age, increasing rapidly after 50 years of age

family history of prostate, [breast](#) or [ovarian cancer](#), especially BRCA1 and BRCA2 gene mutations

a father or brother diagnosed with prostate cancer before the age of 60

There is an association with high testosterone levels.

Diagnosis of prostate cancer

If your doctor suspects you may have prostate cancer, you may have one or more of the following tests:

PSA blood test

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variable, it is common for your doctor to use results from more than one blood test, over time, to help determine your risk of prostate cancer.

Some men with prostate cancer have normal PSA levels, and only one in three men with an elevated PSA level has cancer. As it is not a definitive test, a PSA test is normally used with other tests to diagnose prostate cancer.

There is a lot of confusion around PSA testing. [Click here for more information.](#)

Digital rectal examination

Digital Rectal Examination (DRE) is no longer recommended as a routine test for men who do not have symptoms of prostate cancer. As not all prostate cancers produce high levels of PSA, it may still be used to check the prostate before doing a biopsy.

Biopsy

A biopsy removes small pieces of tissue from different parts of the prostate with the aid of a rectal ultrasound, for examination under microscope. It is used to detect the disease and determine its aggressiveness (the Gleason score of 1-5 is added from two samples to form a score out of 10; low scores of 6 or less, indicate slow growing disease).

Further tests

If cancer is detected in your prostate, you may have other tests such as [MRI](#), [CT](#) or bone scans to see if the disease is contained to the prostate or to help with management and treatment options.

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After finding out you have prostate cancer, you may feel shocked, upset, anxious or confused. These are normal responses. Talk about your treatment options with your doctor, family and friends. Ask questions and seek as much information as you need. It is up to you as to how involved you want to be in making decisions about your treatment.

Learn more about best prostate cancer care:

What should happen next?

This resource can help guide you and your loved ones after your diagnosis.

[GUIDE TO BEST CARE FOR PROSTATE CANCER >](#)

Treatment for prostate cancer

Treatment depends on the extent of the cancer.

Staging

The staging system used for prostate cancer is the [TNM system](#), which describes the

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Active surveillance monitors prostate cancer that is not causing symptoms and is considered low risk (ie cancer is small and slow growing and unlikely to spread). Generally active surveillance involves PSA tests every 3-6 months, rectal examination every 6 months and MRI scans and biopsies at 12 months and 3 years.

Watchful waiting

Watchful waiting is another form of monitoring prostate cancer that involves regular PSA tests and check-ups. Watchful waiting can be suitable for older men where the cancer is not likely to cause a problem in their lifetime.

Surgery

[Surgery](#) with curative intent removes the whole prostate (radical prostatectomy). The main side effects may include impotence and incontinence.

Radiation therapy (radiotherapy)

[Radiation therapy](#) (radiotherapy) is one treatment offered to men with early prostate cancer. It may also be offered if surgery is not an option.

Radiation therapy can be given either with external beam radiation therapy (EBRT) or by implanting radioactive seeds (brachytherapy). Side effects are similar to surgery, however bowel problems may also occur.

Androgen deprivation therapy (hormone therapy)

Some cancers need certain hormones to grow. Prostate cancer needs testosterone.

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ADT can be given with injections or in tablet form. Although no longer commonly used, surgery to remove part or all of the testicles may be preferable in some cases.

Palliative care

In some cases of prostate cancer, your medical team may talk to you about [palliative care](#). Palliative care aims to improve your quality of life by alleviating symptoms of cancer.

As well as slowing the spread of prostate cancer, palliative treatment can relieve pain and help manage other symptoms. Treatment may include radiation therapy, chemotherapy or other drug therapies.

Treatment Team

Depending on your treatment, your treatment team may consist of a number of different health professionals, such as:

GP (General Practitioner) - looks after your general health and works with your specialists to coordinate treatment.

Urologist - specialises in the treatment of diseases of the urinary system (male and female) and the male reproductive system

Radiation oncologist - prescribes and coordinates radiation therapy treatment.

Radiologist - interprets diagnostic scans (including CT, MRI and PET scans).

Medical oncologist - prescribes and coordinates the course of chemotherapy.

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Cancer nurse - assists with treatment and provides information and support throughout your treatment.

Continence care nurse/urology care coordinator - supports people with bladder and bowel management before and after cancer treatment.

Sexual therapist - qualified counsellor who has been trained to help patients manage sexual concerns.

Other allied health professionals - such as social workers, pharmacists, and counsellors

Screening for prostate cancer

There are no tests available with sufficient accuracy to screen populations of men for early signs of prostate cancer. However, early detection and treatment can significantly improve prostate cancer survival.

The test most commonly used to aid early detection of prostate cancer is the prostate specific antigen (PSA) blood test. This is not a diagnostic test as it can only indicate changes in the prostate. If you are concerned about prostate cancer you should talk to your doctor and make an informed choice about whether to have one of the tests designed to find early signs of prostate cancer, in view of the potential risks and benefits.

For more information see our page on [early detection of prostate cancer](#).

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Preventing prostate cancer

There are no proven measures to prevent prostate cancer.

Prognosis for prostate cancer

It is not possible for a doctor to predict the exact course of a disease, as it will depend on each person's individual circumstances. However, your doctor may give you a prognosis, the likely outcome of the disease, based on the type of prostate cancer you have, the test results, the rate of tumour growth, as well as your age, fitness and medical history.

Prostate cancer often grows slowly and even more aggressive types tend to grow more slowly than other types of cancer. If diagnosed early, prostate cancer has one of the highest five year survival rates.

Sources

Understanding Prostate Cancer, Cancer Council Australia © 2024. Last medical review of source booklet: May 2024. We thank the reviewers of this booklet: Prof Declan Murphy, Consultant Urologist, Director – Genitourinary Oncology, Peter

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Understood



**Early detection of
prostate cancer**



**After a diagnosis of
prostate cancer**



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